

# MedCare General Hospital

Department of Clinical Services | Pune, Maharashtra

## Haematology Report — CBC + Iron Studies

MedPlus Diagnostics, Pune | 01 May 2026

|                      |              |                     |                                       |
|----------------------|--------------|---------------------|---------------------------------------|
| <b>Patient Name</b>  | Sneha Patil  | <b>Age / Gender</b> | 32 Years / Female                     |
| <b>Date of Birth</b> | 22 June 1993 | <b>Physician</b>    | Dr. Anil Joshi, MD (General Medicine) |
| <b>MRN</b>           | 50012345     | <b>Report Date</b>  | 01 May 2026                           |

### DIAGNOSES / CONDITIONS

| Diagnosis / Condition                  | Severity               |
|----------------------------------------|------------------------|
| Moderate Iron Deficiency Anaemia (IDA) | Moderate — Hb 8.4 g/dL |
| Menorrhagia — Suspected Cause          | Under Investigation    |

### COMPLETE BLOOD COUNT (CBC)

| Test / Parameter  | Result         | Reference Range      | Status         |
|-------------------|----------------|----------------------|----------------|
| Haemoglobin (Hb)  | 8.4 g/dL       | 12.0-16.0 g/dL       | CRITICALLY LOW |
| Haematocrit (PCV) | 28.2%          | 36-46%               | LOW            |
| RBC Count         | 3.9 million/uL | 4.2-5.4 million/uL   | LOW            |
| MCV               | 71 fL          | 80-100 fL            | LOW            |
| MCH               | 22 pg          | 27-33 pg             | LOW            |
| MCHC              | 29.8 g/dL      | 32-36 g/dL           | LOW            |
| RDW               | 17.2%          | 11.5-14.5%           | HIGH           |
| WBC Count         | 7,200/uL       | 4,500-11,000/uL      | NORMAL         |
| Platelets         | 4,10,000/uL    | 1,50,000-4,00,000/uL | HIGH           |

### IRON STUDIES

| Test / Parameter       | Result    | Reference Range | Status         |
|------------------------|-----------|-----------------|----------------|
| Serum Iron             | 38 ug/dL  | 60-170 ug/dL    | LOW            |
| TIBC                   | 510 ug/dL | 250-370 ug/dL   | HIGH           |
| Transferrin Saturation | 7.5%      | 20-50%          | CRITICALLY LOW |
| Serum Ferritin         | 6 ng/mL   | 20-200 ng/mL    | CRITICALLY LOW |

### PRESCRIBED MEDICATIONS

| Medication       | Dosage | Frequency      | Purpose                                                      |
|------------------|--------|----------------|--------------------------------------------------------------|
| Ferrous Sulphate | 200mg  | 3x daily       | Iron supplement to rebuild iron stores and raise haemoglobin |
| Vitamin C        | 500mg  | With each dose | Significantly improves iron absorption from the tablet       |

IDENTIFIED RISK FACTORS

| Risk Factor              | Explanation                                                                                    |
|--------------------------|------------------------------------------------------------------------------------------------|
| Worsening anaemia        | If heavy periods are not treated, iron loss will continue despite supplementation              |
| Heart strain             | Severe anaemia forces the heart to work harder to compensate for low oxygen delivery           |
| Underlying uterine issue | Heavy periods may indicate fibroids, polyps, or hormonal imbalance — gynaecology workup needed |

PATIENT-FRIENDLY SUMMARY

Patient-Friendly Summary

Sneha Patil is a 32-year-old woman feeling very tired, pale, and breathless because her blood is low on haemoglobin — the protein that carries oxygen around the body. Her iron stores are nearly empty, most likely because of heavy monthly periods over a long time. Blood tests confirm moderate iron deficiency anaemia. She has been prescribed iron tablets three times a day along with Vitamin C to help absorb them. She has also been referred to a gynaecologist to investigate why her periods are so heavy. A follow-up blood test in 8 weeks will check if the iron levels are improving.